The Implementation of Local Wellness Policies in Mississippi Schools

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Learning Outcome: Understand the degree of implementation of physical education/physical activity and health education components of Local School Wellness Policies in Mississippi schools.

The Child Nutrition and WIC Reauthorization Act of 2004 required all schools receiving funding for school meals to implement a Local Wellness Policy. However, there is little to no accountability related to the implementation of the policy. Therefore, the purpose of this study was to evaluate the degree of implementation of physical education/physical activity and health education components of the Local School Wellness policies in Mississippi in 2008. An on-line survey was distributed to 911 public school principals. A total of 635 (70%) surveys were submitted, and a total of 540 surveys were used in data analysis (59.3%). Of the principals that responded to the survey, 79.1% had fully implemented the minimum requirements for Physical Education/Physical Activity, and 65.5% had fully implemented the minimum requirements for Health Education. Additionally, it was reported that 88.73% of all children in grades K-12 were receiving PE curriculum in 2008. Schools are an ideal environment for implementing health intervention programs, but this can be a major challenge for most schools. Therefore, it was encouraging to see a high level of implementation of physical activity and nutrition education requirements in the state of Mississippi, where obesity rates are the highest in the nation. Future research needs to focus on changes to the health of the students in the state and to evaluate if the changes can be related to changes in health requirements in the schools.

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Is the Postpartum Period a Teachable Time for Improving Diet Quality?

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Learning Outcome: To examine the efficacy of a dietary intervention on diet quality in postpartum women.

Background: The American Dietetic Association and the American Society for Nutrition have recently identified the postpartum period as a window of opportunity to modify dietary patterns as mothers are highly motivated to develop healthy eating habits for themselves and their infants. This study was designed to determine the efficacy of a clinic-based behavioral intervention, emphasizing the intake of deep-yellow and dark-green vegetables, on diet quality in postpartum women.

Methods: Two groups of women- 28 breastfeeding and 19 formula feeding, participated in this study. The 4 month-long dietary intervention consisted of one face-to-face counseling session conducted by a dietitian, two follow-up phone calls and 3 pamphlets mailed to the mother's home. Self-efficacy scores; vegetable, vitamin C, and caloric intakes were assessed at baseline and post-treatment.

Results: Self-efficacy scores increased significantly in the breastfeeding group ($P < .01$) and a trend towards improvement was observed in the formula feeding group. There was a significant increase in daily number of servings of all vegetables for both groups ($P < .01$). This increase was partially a reflection of a higher intake of dark-green and deep- yellow vegetables. Vitamin C intake increased slightly in both groups, but the change was not significant. There was a decrease in caloric intake among breastfeeding and formula feeding participants. That decrease approached significance for the formula feeding mothers only. Changes in self-efficacy; vegetable, vitamin C, and caloric intakes were not significantly different between groups.

Conclusion: Dietary intervention appears to be equally effective for breastfeeding and formula feeding postpartum women.

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Dietary Approaches to Stop Hypertension Diet Compliance Decreases Coronary Heart Disease Risk in Overweight and Obese College-Aged Women

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Learning Outcome: Viewers will know how compliance with Dietary Approaches to Stop Hypertension reduces risk of coronary heart disease in overweight and obese college-aged women.

Research Outcome Following Dietary Approaches to Stop Hypertension (DASH) recommendations has resulted in weight loss and improved low-density lipoprotein cholesterol (LDL-C) levels. The Best Foot Forward (BFF) study is a randomized clinical trial that examined the impact of a single or multiple exposures to DASH diet and exercise recommendations in overweight and obese college females at the University of Rhode Island on coronary heart disease (CHD) risk factors. The purpose of the study was to determine if increased DASH diet compliance over eight weeks results in decreased CHD risk in an understudied, at-risk population.

Methods: Fifty-nine overweight/obese college women (ages 18-24 years) completed baseline and post-assessment measures. At each time point, measures included height, weight, waist circumference, three non-consecutive 24-hour dietary recalls using the Nutrition Data for System Research for analysis, two 12-hour fasted blood draws, and blood pressure. The DASH Diet Index was used to measure compliance with DASH recommendations. Subjects were divided into two groups: more compliant (MC) and less compliant (LC).

Results: Mean baseline age was 19.3 ± 1.3 and BMI was 29.6 ± 3.5 kg/m2. At post-intervention, mean DASH score was 7.0 ± 1.8 for MC and was 5.4 ± 1.6 for LC ($p<0.05$). LDL-C decreased significantly more in MC than LC subjects ($p<0.05$). Though overall weight loss was not different between the two groups, the increase in lean mass for MC was borderline greater than LC ($p=0.058$).

Conclusion: Registered dietitians can use DASH recommendations with overweight and obese college females to reduce certain CHD risk factors.

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Women Together for Health: Empowering Women for a Healthier Tomorrow

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Learning Outcome: Describe the components of a comprehensive public health education program for women of childbearing age, discuss results obtained, and apply lessons learned in the Women Together for Health program to future community health initiatives.

Given the obesity epidemic and the associated health problems, improving the American diet and physical activity has become a national health priority. The Women Together for Health (WTFH) program, sponsored by the Maricopa County Department of Public Health, addresses nutrition, healthy weight, physical activity, stress management, and tobacco use. The program is facilitated by a multi-disciplinary team consisting of a Health Educator and Registered Dietitian. Classes are taught in either English or Spanish. The target audience is women of childbearing age with limited education, family income, and employment opportunity. Data measurement consists of pre/post health assessments, pedometers with step-logs, and a short three-month follow-up interview. Throughout 2009, the WTFH had 790 participants and 435 program completers. Women from minority populations comprised 83.3% of participants. The mean age for participants was 34 years old ranging from 11 to 81 years of age. Nearly 82% fell between the childbearing ages of 18-44. Thirty-one percent of participants increased their dairy intake, about 18% switched to low- or non-fat milk from whole, and 53% increased fruit and/or vegetable consumption. The choice of whole grains improved from 59% at pre-test to 78% at post-test, and 47% reported an increase in reading nutrition labels. Ninety percent of the program participants made at least one dietary improvement. Over 64% of participants increased their daily step count by 2,000 steps by the end of the program. Nearly 71% maintained healthy behaviors three months post-program. In conclusion, the WTFH program successfully improved targeted health-related behaviors.

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